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June 16, 2025

Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
200 Independence Ave SW
Washington, DC 20001

Submitted Electronically at www.regulations.gov

Re: Request for Information; Health Technology Ecosystem (CMS-0042-NC) [RIN 0938-AV68]

Dear Administrator Oz,

I am pleased to submit these comments on behalf of the Association for Clinical Oncology (ASCO) in response to the Health Technology Ecosystem Request for Information (RFI) published in the Federal Register on May 16, 2025.

ASCO is a national organization representing more than 50,000 physicians and other health care professionals specializing in cancer treatment, diagnosis, and prevention. We are also dedicated to conducting research that leads to improved patient outcomes, and we are committed to ensuring that evidence-based practices for the prevention, diagnosis, and treatment of cancer are available to all Americans.

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Prior Authorization

The administrative burdens associated with prior authorization contribute to major delays and denials of necessary, appropriate—and in many cases, lifesaving—care. Prior authorization is consistently identified as the largest barrier to care for insured patients. Providers and patients have offered to collaborate on reforms to prior authorization processes, but these calls have largely gone unheard.

The Interoperability and Prior Authorization Final rule (CMS-0057) was a critical step in ensuring that patient care takes priority over paperwork. Far too often necessary medical services are delayed due to excessive and burdensome prior authorization processes. Ensuring there is a single, easy-to-access electronic process in place to file forms and receive approvals within federal health plans

is a welcome change. Timely access to care is critical for people with cancer and ASCO hopes this will help alleviate delays and lessen providers' administrative burden.

However, we are disappointed that the Interoperability and Prior Authorization Final rule does not apply to physician administered drugs—which will continue to be an administrative burden to clinicians and their patients. CMS stated in the proposed rule that drugs were excluded from the interoperability requirements "because the processes and standards for prior authorization applicable to drugs differ from the other 'items and services' for which we propose regulation."[1] However, the National Council for Prescription Drug Programs (NCPDP) Telecommunication Standard Implementation Guide Version D.0 for retail pharmacy drugs to which CMS was referring applies only to Part D drugs, leaving Part B drugs without standards.

Urgent relief is needed now, and the additional delay risks further harming patient access to cancer care. A significant portion of cancer treatment requires patient access to life-saving drugs and the prior authorization requirements associated with these drugs are extremely burdensome for oncologists and their patients. The omission of drugs from this rule is deeply concerning and provides no relief for the provider and their patients, who will continue to experience delays in obtaining necessary therapy.

On page 29 of the display copy of the final rule, CMS states "we will consider options for future rulemaking to address improvements to the prior authorization processes for drugs."[2] We strongly urge CMS to establish an electronic prior authorization process for Part B drugs as soon as possible.

Real Time Benefit Tools

ASCO supports real time benefit tools (RTBTs) that would allow enrollees to view accurate, clinically appropriate, real-time formulary and benefit information. Lack of prescription formulary transparency at the point of prescribing leads to challenges for patients, physicians, and pharmacists at the pharmacy counter when a patient's medication requires prior authorization, is not covered by their plan, or comes with a high co-pay. More concerning, patients facing these challenges may completely abandon treatment, leading to medication adherence issues and worsening health outcomes. Requiring certified vendors to support standard RTBT technology will enable informed conversations between physicians and patients during medication selection and ameliorate many of the problems listed above.

RTBTs allow providers and their patients to compare the patient-specific cost of a drug to that of a suitable alternative, compare prescription costs at different pharmacy locations, view information about out-of-pocket costs, and learn whether a specific drug is subject to utilization management restrictions such as prior authorization, step therapy, or quantity limits. Increasing physician access to these high-

[1] https://www.federalregister.gov/documents/2022/12/13/2022-26479/medicare-and-medicaid-programs-patient-protection-and-affordable-care-act-advancing-interoperability
[2] https://www.federalregister.gov/documents/2024/02/08/2024-00895/medicare-and-medicaid-programs-patient-protection-and-affordable-care-act-advancing-interoperability

value functionalities will address well-known transparency issues and administrative burdens related to drug prescribing and utilization management requirements.

Bulk FHIR Exports

Using Bulk FHIR (Fast Healthcare Interoperability Resources) data exports from Electronic Health Records (EHRs) to CMS (Centers for Medicare & Medicaid Services) for clinical quality data submissions presents a transformative opportunity with significant pros and cons. While the concept of leveraging Bulk FHIR data exports for CMS quality data submission offers theoretical advantages, ASCO has significant reservations regarding its immediate feasibility and potential implications.

Arguably the most notable benefit is to reduce provider burden. Automated extraction and submission of large datasets would drastically reduce the manual effort, chart abstraction, and administrative time currently spent by providers on quality reporting. Instead of generating custom reports for each quality measure, EHRs could export data in a standardized FHIR format, making the process more efficient and less disruptive to clinical workflows. Data submitted via Bulk FHIR could potentially be leveraged for multiple quality programs and reporting requirements, further minimizing redundant efforts. Bulk FHIR can facilitate more frequent data updates, allowing providers to monitor performance and identify care gaps more proactively, rather than waiting for retrospective reporting cycles.

In theory, Bulk FHIR could improve data quality and accuracy. FHIR provides a standardized way to represent healthcare data, leading to greater consistency and reducing errors that arise from disparate data formats. With automated extraction, there's a higher likelihood of capturing all relevant data elements for a given quality measure. Eliminating manual data entry and manipulation significantly lowers the risk of human error. CMS could gain a more comprehensive and granular understanding of population health trends, allowing for better policy decisions and targeted interventions. Access to standardized, bulk data would allow CMS to perform quicker and more sophisticated analyses of quality performance across the healthcare system. With richer data, CMS could provide more meaningful benchmarks to providers, helping them identify areas for improvement.

The availability of standardized, granular data could also facilitate the development of more sophisticated and outcome-focused quality measures. Bulk FHIR could be the foundational component for the shift to digital quality measures, enabling the creation of fully electronic, computable measures. Medical societies have the expertise and access to providers to develop evidence-based clinical quality measures; however, they experience substantial challenges accessing the data CMS has via the ResDAC (Research Data Assistance Center). It is laborious and very expensive. A more efficient and less costly system would need to be put in place.

Despite the potential to reduce administrative burden and facilitate digital quality measures, ASCO is not confident that Bulk FHIR data exports should be provided to CMS for quality reporting for data security and privacy reasons. Transferring massive amounts of sensitive Protected Health Information (PHI) in bulk increases the attack surface for cyber threats and data breaches. Robust encryption, access

controls, and compliance with HIPAA are paramount. While FHIR supports de-identification, ensuring complete de-identification of large, complex datasets while maintaining data utility for quality measurement can be challenging. While certified EHRs are mandated to support FHIR APIs, the full capabilities for bulk export and the robustness of these implementations can vary. Some EHR vendors may require significant upgrades or development. Both CMS and providers need substantial IT infrastructure, including high-performance servers, robust network bandwidth, and storage capacity to handle the volume of data. This represents a significant financial investment. Integrating Bulk FHIR exports with existing reporting systems and data warehouses can be complex, especially for legacy systems.

Data quality and standardization challenges persist. FHIR standardizes the format of data, but it doesn't automatically fix underlying data quality issues within the EHR. If the data within the EHR is inconsistent, incomplete, or poorly documented, the FHIR export will reflect those issues. While FHIR aims for standardization, variations in how clinical concepts are captured and coded across different EHRs may still require significant effort in data cleaning, mapping, and transformation to ensure comparability for quality measure calculation. Ensuring clinicians consistently document data in a structured, FHIR-compliant way within their daily workflows is crucial, but can be a persistent challenge.

There are implementation complexities that require technical expertise. Organizations need skilled personnel with expertise in FHIR standards, data management, security, and system integration to effectively implement and maintain Bulk FHIR pipelines. Establishing clear governance frameworks for data sharing, consent management, and ensuring the ethical use of large-scale health data is critical. While designed for bulk data, processing and transferring extremely large datasets can still lead to latency and performance issues if systems are not optimally designed and scaled.

Should CMS move forward with bulk FHIR data exports, it is imperative to conduct pilot testing with EHR vendors to assess the current state of data quality and the true feasibility of this centralized model. CMS must also consider the impact on small and rural practices. Their capacity for technological adoption and integration often lags behind larger institutions. A clear plan would be needed to support these practices in transitioning to more modern data submission methodologies.

Interoperability and Quality Reporting

Investing in robust FHIR-based APIs and ensuring data aligns with the United States Core Data for Interoperability (USCDI) are important steps in improving interoperability and quality reporting. FHIR provides the "how" for data exchange, and USCDI defines the "what" (the core data elements). When clinical data is consistently captured and exchanged using FHIR and USCDI, it becomes much more amenable to automated extraction and calculation for quality measures. This eliminates the need for manual chart abstraction or bespoke data transformations for each reporting program. If an EHR system is truly interoperable and can reliably export patient data in a standardized, computable format, then the effort to generate quality reports shifts from data gathering and formatting to simply running predefined queries against that standardized dataset.

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We appreciate the opportunity to comment on the Health Technology Ecosystem request for information, and we look forward to working with the agencies on efforts to improve efficiency and health technology for cancer patients and providers. Please contact Gina Hoxie (gina.hoxie@asco.org) with any questions or for further information.

Sincerely,

Lynn M. Schuchter, MD, FASCO
Chair of the Board
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